HEALTHCARE ANALYTICS / EXECUTIVE BRIEF

Medicaid prescription spending Where the dollars go

Mohammed Mia | Python + SQL | 2024

$27.54B
Reported reimbursement
Before manufacturer rebates

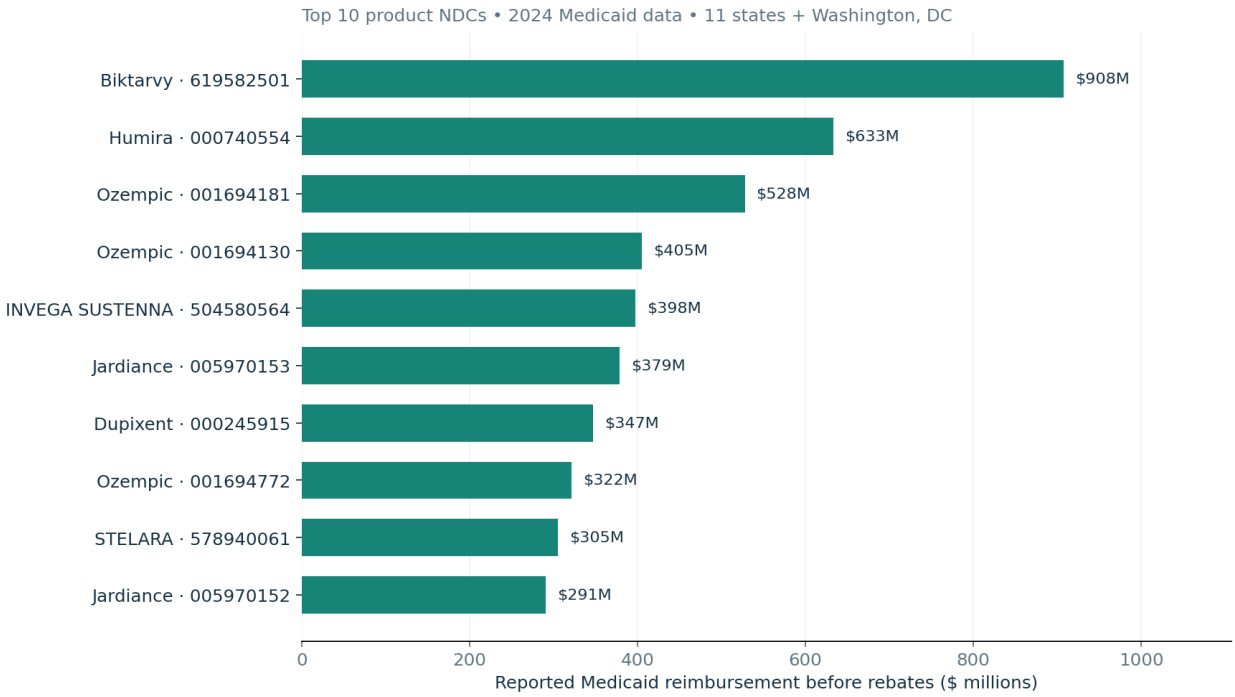
12
Jurisdictions analyzed
11 states + Washington, DC

37.1%
Top 50 product share
Of reported reimbursement

The question I wanted to answer

Which products account for Medicaid prescription spending, and where do comparable formulations have different reimbursement per unit? I built a Python and SQL workflow to prepare the data, analyze concentration, and identify products for further price review.

Where observed prescription spending concentrates



Product-level NDCs combine package sizes, not all strengths of a brand. Suppressed reimbursement is unavailable.

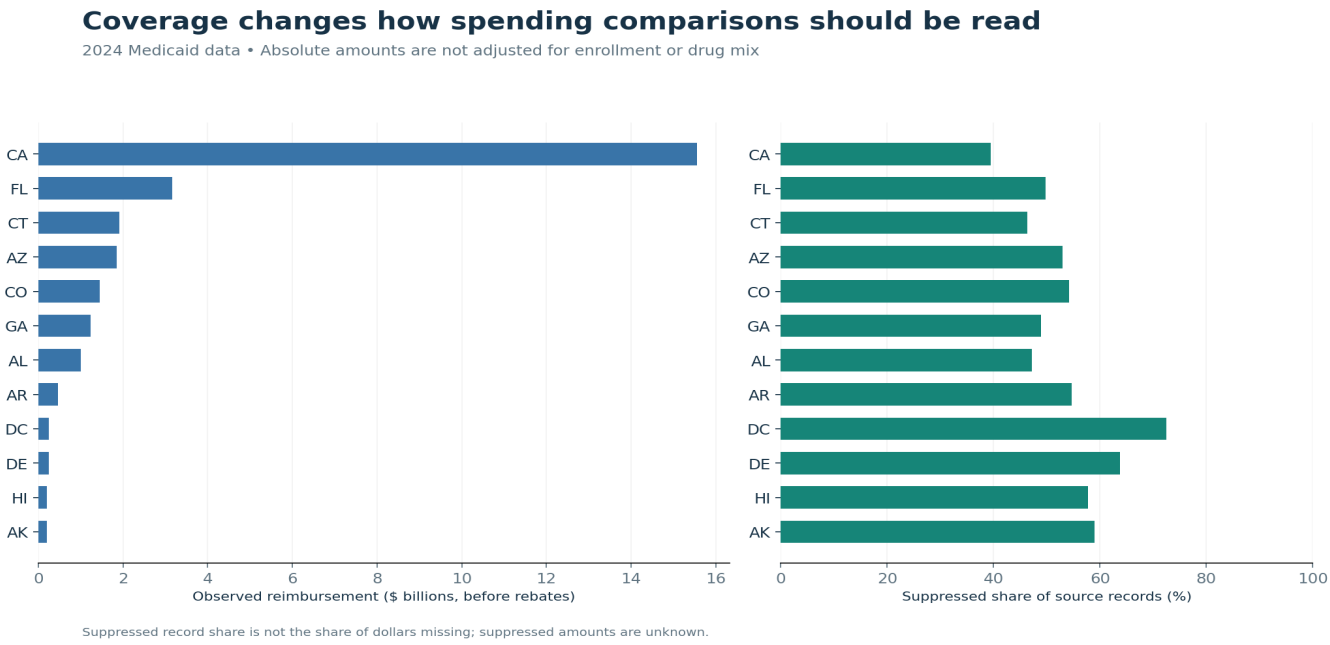
Biktarvy product NDC 619582501 contributes $907.89M. The top 10 product NDCs account for 16.4% of reported reimbursement. Product keys combine package sizes, not every strength of a brand.

What I would prioritize

Start detailed reviews with the concentrated high-dollar product set. The top 50 product NDCs account for 37.1% of reported reimbursement, providing a focused starting point for healthcare finance analysis.

Comparing spending and prices

I compare reported amounts alongside source coverage, then examine recorded formulation matches. The calculations support review priorities; clinical substitutability and net savings need separate evidence.



California contributes 56.5% of this dataset's reimbursement. This is an absolute spending share, without enrollment or drug-mix adjustment. Suppressed record percentages do not estimate missing dollar shares.

How I compare recorded formulations

I match ingredient, strength, strength unit, dosage form, and route within the same jurisdiction, quarter, and utilization type. I compare NDA- and ANDA-listed prescription products using total reimbursement divided by total units, with a marketing-date overlap check.

Across 562 formulations and 6,675 matched reporting groups, positive differences sum to a $516.16M illustrative gross reimbursement gap. The signed aggregate, including negative differences, is $506.73M.

This is a price-review screen, not achievable or net savings. Application categories do not fully define brand/generic status. Rebate effects, reimbursement unit types, therapeutic equivalence, patient suitability, and practical availability remain unverified.

Data and calculation notes

The extract covers 11 states and DC. It contains 1,048,575 utilization records, including 543,945 with suppressed amounts. I leave those amounts missing. Product descriptions cover 99.42% of reported dollars; unmatched spending remains in totals. Tests check joins, eligibility, weighting, and formulation matching.

Sources and acknowledgments

Sources: CMS 2024 utilization data, FDA NDC Directory, CMS reimbursement definitions. See the repository methodology for scope and matching details.

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